

Glaucoma in Pregnancy for Patients

by the American Glaucoma Society Patient Engagement Committee

When do I need to talk with my doctor about pregnancy?

Talk with your eye doctor **before you plan to become pregnant.**

- If your pregnancy is unplanned, tell your eye doctor as soon as you find out you are pregnant.
- Glaucoma can run in families. Your doctor may be able to help you know your baby's risk of glaucoma.
- Most patients do well during pregnancy. However, there are some glaucoma medications that are not recommended to be taken during parts of pregnancy.
- In some cases, your doctor might suggest that you have a laser treatment or surgery before trying to become pregnant, or during pregnancy. This may decrease the number of glaucoma medications you need during pregnancy and breastfeeding.

Do I need to change my glaucoma drops during pregnancy?

There is not a lot of information about the safety of glaucoma medications during pregnancy. **Many patients can continue their eyedrops, but please talk with your eye doctor and obstetrician about your specific case.**

- You can decrease the amount of glaucoma medication that is absorbed by your body by doing "punctal occlusion". This means **pressing your finger on the skin between the inner corner of your eye and your nose for a minute** after you use your drops.

Do I need to change anything about how I deliver my baby?

You can deliver your baby the way you and your obstetrician decide. From a glaucoma standpoint, **vaginal birth and Caesarean section are both ok.**

- Most doctors recommend not using brimonidine late in pregnancy near the time of delivery.

Do I need to change my glaucoma drops while breastfeeding?

You should not use brimonidine while breastfeeding. It can be dangerous for your baby and can make your baby stop breathing.

- If possible, use your glaucoma drops right after breastfeeding or pumping, not before.

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